

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 2 Open-label Randomized Study of V940 in Combination With BCG Versus BCG Monotherapy in Participants With High-risk Non-muscle Invasive Bladder Cancer (INTerpath-011)
Short Title:	A Clinical Study of Intismeran Autogene (V940) and BCG in People With Bladder Cancer (V940-011/INTerpath-011)
Protocol Number:	648098527540966882
NCT Number:	NCT06833073
Sponsor Name:	Merck
Investigational Product:	BCG vaccine
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. BCG Arms: Event-free Survival (EFS): EFS is defined as the time from randomization to any of the following events, as determined by blinded independent central review (BICR) where applicable: High-grade (HG) non-invasive papillary carcinoma (Ta) or carcinoma in situ (CIS) in the bladder at the 24-week assessment or later; Any T1 stage disease in the bladder; Any T2 stage or greater in the bladder, including transurethral prostate stromal invasion of urothelial carcinoma (UC); High-risk disease (defined as HG Ta, CIS, ?T1) of the urethra or upper tract (ureters, renal pelvis); Metastatic UC [defined as regional lymph node metastasis of UC (stage N1 or greater), or distant lymph node or visceral metastasis of UC (stage M1)]; Or death due to any cause. The EFS for BCG-treated participants will be presented. 2. Intismeran autogene Monotherapy Arm: Complete Response Rate (CRR): CRR is defined as the percentage of participants who achieve a Complete Response (CR) defined as the absence of all of the following as determined by BICR using urine cytology, biopsy, and radiology assessments as applicable, and local cystoscopy evaluation: High-risk non-muscle invasive UC (defined as HG Ta, CIS, or any T1 disease of the bladder, urethra, or upper tract [ureters, renal pelvis]); Any T2 or greater in the bladder, including transurethral prostate stromal invasion of UC; Metastatic UC [defined as regional lymph node metastasis of UC (N1 or greater), or distant lymph node or visceral metastasis of UC (M1)]. The CRR for intismeran autogene monotherapy-treated participants will be presented.
Secondary Obj.:	1. BCG Arms: 12-Month Event-free Survival (EFS) 2. BCG Arms: 24-Month Event-free Survival (EFS) 3. Recurrence-free Survival (RFS)

2.2 Background & Rationale

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

Researchers are looking for new ways to treat people with high-risk non-muscle invasive bladder cancer (HR NMIBC). NMIBC is cancer in the tissue that lines the inside of the bladder but has not spread to the bladder muscle or outside of the bladder. High-risk means NMIBC may have a high chance of getting worse or coming back after treatment. HR NMIBC can also include carcinoma in situ (CIS). CIS is bladder cancer that appears flat and is only in the inner layer (surface) of the bladder. CIS is not raised and is not growing toward the center of the bladder.

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE2, Status: RECRUITING

Target N: 308

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

The main inclusion criteria include but are not limited to the following:

✓ Is an individual whose most recent TURBT was performed within 12 weeks before randomization/allocation and showed BICR-confirmed high-risk NMIBC histology

BCG Arms:

- * Has high-risk non-muscle invasive (HG Ta, T1, and/or CIS) UC of the bladder
- * Is BCG-naïve defined as either having never received BCG or having received BCG more than 2 years before high-risk NMIBC recurrence. Recurrence must be at least 24 months from the last exposure to BCG with evidence of complete response during the 2-year period post BCG

Intismeran autogene Monotherapy Arm:

- * Has CIS +/-papillary non-muscle invasive UC of the bladder
- * Is ineligible for, or refusing, any IVESIC therapy
- * Is either BCG-naïve (as defined above) or BCG-exposed but did not receive protocol-specified minimum dosing of BCG and experienced recurrence of high-risk NMIBC within 2 years of the last dose of BCG
- * Human immunodeficiency virus (HIV)-infected individuals must have well controlled HIV on antiretroviral therapy (ART)

4.2 Exclusion Criteria

The main exclusion criteria include but are not limited to the following:

- * Has a history of or concurrent locally-advanced (ie, T2, T3, T4) or metastatic UC
- * Has concurrent extravesical (ie, urethra, ureter, renal pelvis) non-muscle invasive UC or a history of extravesical non-muscle invasive UC that recurred within the last 2 years, with certain exceptions
- * Has a known additional malignancy that is progressing or has required active treatment within the last 3 years
- * Has had a myocardial infarction within 6 months of randomization/allocation

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

- * Has received any systemic anticancer therapy including investigational agents within 4 weeks before randomization/allocation
- * Has received prior treatment with a cancer vaccine
- * Has immunodeficiency or is receiving chronic systemic steroid therapy
- * Has active autoimmune disease that has required systemic treatment in the last 2 years
- * Has any contraindication to IV contrast and gadolinium or is otherwise unable to have imaging with either computerized tomography urogram (CTU) or Magnetic resonance urography (MRU)

BCG Arms:

- * Has current active tuberculosis
- * Has a known history of HIV infection

Intismeran autogene Monotherapy Arm:

↳ HIV-infected individuals with a history of Kaposi's sarcoma and/or Multicentric Castleman's Disease

11. Study Identifiers & Governance

Primary ID: 648098527540966882

Registry Number: NCT06833073

Sponsor: Merck

Study Title: A Clinical Study of Intismeran Autogene (V940) and BCG in People With Bladder Cancer (V940-011/INTerpath-011)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____